

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Angela Brooks
Date of birth	March 15, 1967
Age at visit	58 years
Sex	Female
MRN	PAD-33018
Address	414 East Davis Boulevard, Tampa, FL 33606
Phone	(813) 555-0193
Emergency contact	Marcus Brooks (spouse), (813) 555-0196
Primary care	Dr. Lila Ortega, MD, (813) 555-0120
Preferred language	English

2. Insurance

Primary insurance

Payer	Palm Mutual of Florida, Choice PPO
Member ID	PM44821009
Group	PM-TPA-22
Prior authorization	Required for peripheral stent placement
Auth telephone	1-800-555-0177

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I70.212	Atherosclerosis of native arteries of extremities with intermittent claudication, left leg	2025	Active
F17.210	Nicotine dependence, cigarettes, uncomplicated	2004	Active
E78.5	Hyperlipidemia, unspecified	2019	Active
I10	Essential hypertension	2017	Active

4. Current medications

Medication	Dose	Frequency	Indication
Aspirin	81 mg	Daily	Antiplatelet
Atorvastatin	40 mg	Nightly	Hyperlipidemia

Medication	Dose	Frequency	Indication
Lisinopril	10 mg	Daily	Hypertension

Cilostazol has not been prescribed. She takes aspirin as her antiplatelet.

5. Allergies

Allergen	Reaction	Severity
No known drug allergies	Not applicable	Not applicable

6. Visit notes — vascular surgery

Visit date: September 10, 2025 | **Attending:** Priya Raman, MD, FSVS | **Type:** New patient consult

Chief complaint. Left calf pain at one block, limiting her mail route.

History of present illness. Angela Brooks is a 58-year-old mail carrier. For four months her left calf tightens after one block and she has to stop. The pain restricts her employment: she can no longer finish her walking route without two extra breaks, and she has asked to be moved to a driving route. She has no rest pain, no foot ulcer, and no gangrene. She smokes one pack of cigarettes daily and has done so for twenty years. She has not been in a walking program. She takes aspirin 81 mg daily. She does not take cilostazol.

Examination.

Blood pressure 138/84 mmHg	Smoking One pack daily, current
Left foot Warm. No ulcer. No dependent rubor. No rest pain by history.	Pulses Left femoral 2+. Left pedal pulse diminished. Right foot pulses normal.
Walking limit One block, left calf. Less than two blocks.	

Assessment and plan.

Lifestyle-limiting left-calf claudication. Not critical limb ischemia.
Order left ABI and CTA of the aorta and runoff.
Start a structured walking program today, September 10, 2025: walk to the onset of calf pain, rest, and repeat, 40 minutes a day, five days a week. Continue aspirin. Smoking cessation was discussed at this visit. She declined nicotine replacement and declined varenicline. She continues to smoke.
No stent is ordered today. Reassess after the walking program.

Electronically signed: Priya Raman, MD, FSVS — September 10, 2025 at 9:10 AM

Visit date: October 1, 2025 | **Attending:** Priya Raman, MD, FSVS | **Type:** Follow-up

Chief complaint. She wants a stent. The walking program has lasted three weeks.

History of present illness. She returns three weeks after the walking program started. She has completed three weeks of the walking program, from September 10, 2025 through October 1, 2025. Calf pain still starts at one block, and she is still unable to finish the mail route. She is still smoking one pack daily. She is still on aspirin and is not on cilostazol. CTA and ABI are reviewed. She asks for a stent now because her employer has set an October deadline for the route change. There is still no rest pain, no ulcer, and no gangrene.

Examination.

Walking program Started September 10, 2025. Duration today is three weeks, not twelve.	Symptom change Unchanged. Still limited to one block.
Smoking Continues, one pack daily. Cessation counseling was given September 10, 2025.	Antiplatelet Aspirin 81 mg daily. No cilostazol.
Tissue loss None	

Assessment and plan.

Left superficial femoral artery stenosis of 80 percent with ABI 0.68. Anatomy would be a stent target if the indication were complete.

The indication in use is claudication, not critical limb ischemia. The structured walking program has lasted three weeks. A twelve-week trial is not documented.

Despite the short trial, she requests scheduling. Order left femoropopliteal stent, CPT 37226.

Electronically signed: Priya Raman, MD, FSVS — October 1, 2025 at 3:30 PM

7. Diagnostic reports

Ankle-brachial index — September 12, 2025

Ordering physician	Priya Raman, MD, FSVS
Facility	Gulfshore Vascular Lab
Right ABI	1.02
Left ABI	0.68

Interpretation. Left ankle-brachial index is 0.68, which is at or below 0.90. Right ankle-brachial index is normal. Toe pressures were not required because the ankle index is diagnostic.

Electronically signed: Priya Raman, MD, FSVS — September 12, 2025

CTA abdomen and pelvis with runoff — September 18, 2025

Ordering physician	Priya Raman, MD, FSVS
Facility	Bayshore Imaging
Indication	Left calf claudication
Contrast	Intravenous, no reaction

Interpretation. Infrarenal aorta and both iliac arteries are patent, with no stenosis of 50 percent or more. The mid left superficial femoral artery has an 80 percent stenosis over a 6 cm length. The left popliteal artery is patent. Tibial runoff is patent. The right superficial femoral artery has a 30 percent plaque and is not a target. Impression: 80 percent stenosis of the mid left superficial femoral artery. Iliac arteries are not a stent target.

Electronically signed: Owen Clarke, MD — September 18, 2025

8. Planned procedure

Planned procedure	Left femoropopliteal stent of the superficial femoral artery
CPT	37226 — femoral or popliteal revascularization with stent, unilateral
Laterality and vessel	Left superficial femoral artery
Not ordered	Iliac stent, lithotripsy, bypass, and tibial intervention are not ordered
Surgeon	Priya Raman, MD, FSVS
Payer	Palm Mutual of Florida
Authorization status	Not submitted. Request dated October 1, 2025.

Clinical indication summary

Lifestyle-limiting claudication at one block, restricting her work as a mail carrier. No rest pain, ulcer, or gangrene.

Left ABI 0.68 on September 12, 2025.

CTA September 18, 2025: 80 percent stenosis of the mid left superficial femoral artery. Iliac arteries are patent.

Structured walking program from September 10, 2025 through October 1, 2025, a duration of three weeks. Twelve weeks are not documented.

Aspirin 81 mg daily is the antiplatelet. Cilostazol has not been used. She still smokes. Cessation counseling occurred on September 10, 2025.

This document is for clinical and administrative demonstration only. Patient identifiers are fictional.